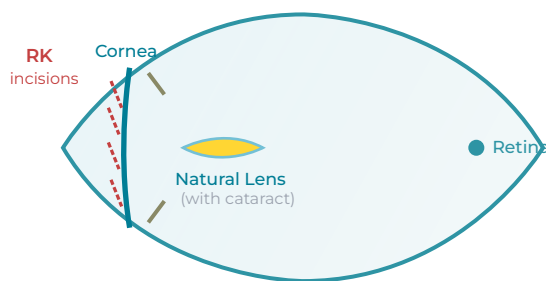


Cataract Surgery After Radial Keratotomy (RK)

What you need to know about cataract surgery when you have had RK in the past

If you had radial keratotomy (RK) years ago and are now developing cataracts, you are not alone. RK was a popular vision correction procedure in the 1980s and 1990s. It helped many people see without glasses at the time. But those tiny incisions in the cornea never fully heal, and they create unique challenges when it comes time for cataract surgery. **The good news: with the right expertise and planning, excellent outcomes are absolutely achievable.**



Cross-section showing RK incision scars on the cornea affecting light entry

Why RK Makes Cataract Surgery Different



Vision That Shifts

RK corneas change shape throughout the day. Your vision may be different in the morning versus the evening, making measurements tricky.



Progressive Change

Over decades, many RK patients become more farsighted as the cornea continues to flatten along the old incision lines.



Lens Calculation

Standard formulas assume a smooth cornea. RK creates irregular surfaces that make choosing the right lens implant more complex.

What You Should Expect

Post-RK cataract surgery requires more planning and precision than a routine case. The accuracy of lens calculations in post-RK eyes is lower than in normal eyes. That is not anyone's fault. It is simply how these corneas behave.

Most post-RK patients achieve excellent vision. But the path may involve more fine-tuning than it would for someone with a normal cornea. Sometimes a lens exchange or an additional "piggyback" lens is needed to dial in the final result. Knowing this upfront leads to better outcomes and less frustration.

How Dr. Tokuhara Manages Post-RK Eyes

- 1 Multiple Measurements**
Different machines capture corneal data differently. Using multiple devices reveals the most accurate picture of your unique cornea.
- 2 Advanced Lens Formulas**
Standard IOL formulas do not work well for post-RK eyes. Specialized calculations using both central and surrounding corneal zones improve accuracy.
- 3 Real-Time Verification**
Intraoperative aberrometry measures the eye during surgery itself, giving a real-time check on the lens power before the procedure is complete.
- 4 Cross-Referencing Results**
Multiple formulas are compared against each other. When they agree, confidence is high. When they disagree, additional analysis narrows the best choice.
- 5 Honest, Upfront Planning**
If a staged approach (lens exchange or piggyback lens) may be needed, we discuss that before surgery, not after. Planning for adjustment is smart, not a sign of failure.

A Rare But Important Possibility

In uncommon cases, the cornea can split along old RK incision lines during surgery. These areas were permanently weakened decades ago. If this happens, corneal sutures are placed during the procedure to close the opening. The surgery can still be completed successfully, but the sutures may extend recovery time compared to a routine case. This is rare, but knowing it is possible helps you feel prepared rather than surprised.

Questions to Bring to Your Consultation

- How many post-RK cataract surgeries have you performed?
- Which IOL calculation methods will you use for my eye?
- Will you use intraoperative aberrometry during my surgery?
- What happens if my lens power needs adjustment after surgery?
- Do you have experience managing corneal complications in RK eyes?
- What type of lens implant do you recommend for my situation?

Ready to Discuss Your Options?

Every post-RK eye is unique. A thorough evaluation with the right measurements is the first step toward clear vision again.

Schedule a Consultation: 760.340.4700

drtokuhara.com/complex-cases/previous-rk

